The last mile of a diabetes result.

The lab calls the doctor. Nobody calls the patient.

A closure ledger for abnormal and critical diabetes results. It records the named owner, the stage clock, the clinician's one tap, the nurse's read-back, the patient or caregiver contacted, the attempts that failed, the next administrative step, and a public receipt.

THE PROBLEM IN ONE LINE

The lab is finished when it delivers. The clinic is not finished when it clicks acknowledge. The patient-side last mile is unowned.

25.8%

of 1,187 critical values reached the treating clinician in an Indian tertiary hospital. 52.9% went to nursing staff. 90% of never-notified values were outpatients.

Agarwal et al., Indian J Clin Biochem 2015 (PMC4310845).

01 THE GAP IS NOT NOTIFICATION. IT IS THE LAST MILE.

The 2015 audit found a written policy, a log book and a read-back rule. The failure was still the last mile: the result reached a desk, not the person who could act. **14% of positive results never returned** in an Indian secondary-care setting (NMJI 2015). A systematic review found **6.8% to 62% of abnormal ambulatory results were not followed up** (J Gen Intern Med 2012). The existing lab tools stop at clinician acknowledgement. The patient-side attempt, the unreachable outcome and the next step are the unclaimed record.

Four surfaces make the closure honest.

01 ONE TAP

The clinician sees one line and taps once. A wrong printed value is corrected with a reason, and the correction is in the chain.

02 READ-BACK

The nurse records the value read back and what the patient understood before an informed outcome.

03 IDENTITY HOLD

An unmatched report cannot be assigned. Two attempts at least 20 hours apart before an unreachable close. Never a false informed.

04 REGISTER + RECEIPT

The quality head prints the register, sees the exception, and closes the loop. The public receipt proves order, not truth.

01 LAB RELEASE	02 ONE TAP	03 READ-BACK	04 REGISTER
The clock starts at release, not at a WhatsApp folder.	The clinician sees one line. No form.	Nurse records the value and what the patient understood.	Exception visible; A4 print; the loop closes.

03 WHY THE DIABETES TRACK IS THE RIGHT FIRST MARKET.

The first protocol is the check a diabetes drug makes necessary: start or titrate an ACE inhibitor or ARB, then potassium and eGFR at two to four weeks. The same receipt covers SGLT2i, MRA, annual uACR/eGFR, a critical potassium, or an abnormal HbA1c. **101 million people in India live with diabetes** (ICMR-INDIAB-17, 2023). The clinician sets the protocol; the lab flag sets criticality; the product never infers a screening deadline or a clinical action.

What we ran, what we have not, and what we need.

A baseline audit, not a demo.

04 MEASURED, AND DELIBERATELY NOT CLAIMED.

WHAT WE RAN

212 unit and integration tests across 33 files.

19 browser checks across four specs, desktop and mobile.

12 / 12 database isolation checks.

5 obligation browser checks: one tap, read-back, register, intake, role boundary.

20 synthetic diabetes receipts across 10 patients and 4 labs; 4 role homes.

WHAT WE HAVE NOT VALIDATED

- No buyer interview, baseline audit, paid pilot or LOI.
- No live LIS, HL7/FHIR or WhatsApp send. The demo imports a CSV lab feed.
- No consent capture, DPA, retention runbook or independent security review.
- No external anchor. The chain is tamper-evident inside one database.
- No clinical validation. No diagnosis, triage, risk score or treatment advice.
- No NABL or NABH certification. The certificate maps evidence and leaves the verdict to the assessor.

05 THE BUYER AND THE PILOT.

The payer is the lab chain quality head or a hospital group quality cell; the user is the diabetes clinic; the champion is the endocrinologist or quality head. The lab already sends the report. It pays for proof that the last mile closed and for a differentiator to referring doctors. **Pilot: ₹60,000 for 90 days. One lab, one diabetes OPD, one ward. 100 de-identified reports for the baseline. Continuation is a signed decision, not a survey answer.**

THE ASK

Three sites. 100 consecutive abnormal or critical diabetes reports each. Eight weeks. A baseline audit, the closure workflow, then a paid continuation. If the first two gates fail, the thesis stops.

What we need: one lab or OPD sponsor, one practising clinical lead, and 100 de-identified reports. What we will report: closure rate within 7 days, documented unreachable, zero false informed, and next-step completion.

06 THE LINE THAT MATTERS.

The lab is done when it delivers. The clinic is done when the patient is reached.

The result nobody told.

07 WHAT WOULD MAKE US STOP.

BASELINE	CONCIERGE	CONTINUATION
100 consecutive reports per site. Count the results with no documented owner, no patient contact and no critical read-back. If the gap is under 30%, the site does not have the problem we are built for.	20 receipts in week one. Measure staff minutes per report and audit every informed row against the phone record. Stop on any false informed or more than 3 minutes per report.	Eight weeks. A signed paid continuation, not a survey answer. Closure rate within 7 days, unreachable documented, next step completed. No signature means stop.

A pilot that cannot produce these numbers is a demo.

Sources: Agarwal et al., Indian J Clin Biochem 2015 (PMC4310845); Ganiyari et al., NMJI 2015; J Gen Intern Med 2012 systematic review; ICMR-INDIAB-17, Lancet Diabetes & Endocrinology 2023. Product measurements are from the current build. The receipt and patient records in this document are synthetic.

Synthetic data. Administrative only. No clinical interpretation. This is a product and validation document, not legal or clinical advice.